

# The Athletic Trainer Gap

## Youth Health and Safety in Michigan's Rural School Districts

*A Nonpartisan Policy Research Brief*

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## Executive Summary

One-third of American high schools with athletics programs have no access to a certified athletic trainer. In rural school districts, the gap is significantly wider: suburban schools are 3.55 times more likely to have athletic training services than rural schools, and large schools are 18.5 times more likely than small schools to employ a trainer. Only one state, Hawaii, requires public high schools to employ athletic trainers.

In Michigan, rural districts in the Thumb, Saginaw Valley, and northern Lower Peninsula face compounding barriers: limited budgets, distance from hospitals and clinics, small school size, and misconceptions about the role of athletic trainers. The result is that student-athletes in these communities practice and compete without the on-site medical professional specifically trained to prevent, evaluate, and manage athletic injuries, including concussions, cardiac events, and heat-related illness.

This brief examines the scope of the athletic trainer gap, its consequences for student-athlete health and safety, the behavioral health dimension that current research increasingly identifies, and the policy considerations relevant to Michigan legislators and school administrators.

*Key Finding: Nationally, only 56% of high schools employ an athletic trainer, a decline of 10 percentage points since 2017. Rural and inner-city schools have far less access than suburban schools. In rural settings, emergency medical response times average 13.6 minutes compared with 7.0 minutes in urban locations, and victims in rural areas face seven times the risk of death when emergency response exceeds 30 minutes. Athletic trainers in these settings may be the only health care provider young athletes regularly encounter.*

## 1. The Scale of the Gap

### 1.1 National Data

The Athletic Training Locations and Services (ATLAS) Project, a joint initiative of the Korey Stringer Institute and the National Athletic Trainers' Association, represents the most comprehensive dataset on athletic training coverage in American secondary schools. ATLAS data from 20,272 public and private secondary schools found that 66% of schools received some form of athletic training services, while 34% had none.

The geographic disparity is stark. Suburban schools reported the highest access rate at 80.1%. Rural schools reported significantly lower access, with suburban schools demonstrating 3.55 times greater odds of having athletic training services than rural schools. School size compounds the effect: large schools (enrollment above 1,319 students) had 18.5 times greater odds of athletic training access than small schools

(enrollment below 293 students). The schools most likely to lack coverage are simultaneously the smallest, most rural, and most resource-constrained.

## **1.2 Michigan Context**

Michigan's secondary school landscape includes a significant number of small, rural districts in the Thumb, Saginaw Valley, and northern Lower Peninsula. These districts mirror the national pattern: limited budgets and remote locations create compounding barriers to athletic trainer employment. In interviews conducted by Pike et al. (2017), an athletic director at a Michigan public school directly cited lack of funding as the primary barrier, stating the school simply did not have the funding available for an athletic trainer.

Athletic directors across the country identify three primary barriers to hiring athletic trainers: budgetary constraints, rural location without nearby hospitals or clinics, and misconceptions about the role of athletic trainers that lead administrators to believe that first-aid-trained coaches provide adequate medical coverage. A fourth barrier, identified specifically in public schools, is that hiring decisions are controlled by superintendents and school boards rather than athletic directors, meaning the people closest to the need often lack the authority to address it.

## **2. Health and Safety Consequences**

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### **2.1 Injury Response in Rural Settings**

The absence of athletic trainers in rural schools creates a direct risk to student-athlete safety. Research on urban-rural differences in emergency medical response found that mean emergency response time in rural locations was 13.6 minutes, nearly double the 7.0-minute urban average. Scene time and transport time were also significantly longer in rural settings. Most critically, victims in rural areas face seven times the risk of death when emergency response exceeds 30 minutes.

Athletic trainers provide immediate, on-site medical response for injuries including concussion, cardiac arrest, heat-related illness, and musculoskeletal trauma. In rural schools without a trainer, the first response often falls to coaches whose first-aid training does not include the diagnostic and triage capabilities of a certified athletic trainer. The gap between what a coach can provide and what a student-athlete needs in an acute medical event is the gap that costs lives.

### **2.2 Concussion Management**

Concussion identification and management is one of the most critical functions athletic trainers serve in secondary schools. Without an athletic trainer, concussion protocols depend on coaches who may lack training in recognizing post-concussive symptoms, may face competitive pressure to return athletes to play prematurely, and may not have

relationships with medical providers who can conduct formal concussion evaluations. Rural schools without nearby neurological or sports medicine specialists face additional barriers to appropriate follow-up care.

### **2.3 The Behavioral Health Dimension**

Emerging research identifies athletic trainers as front-line observers of student-athlete behavioral health. Trainers who see athletes daily in treatment rooms, practice settings, and competitions are often the first professionals to notice changes in mood, withdrawal, appetite, sleep disruption, or substance use. In schools without counselors, social workers, or psychologists on staff, which disproportionately describes rural Michigan districts, the athletic trainer may be the only health care provider a young athlete regularly encounters.

This dimension connects the athletic trainer gap directly to the broader youth behavioral health access crisis documented in NCI's PlayStrong research. Where no athletic trainer exists, one of the most natural observation and referral points for adolescent behavioral health is simply absent from the school ecosystem.

## **3. Existing Policy Landscape**

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Only one state, Hawaii, mandates that public high schools employ athletic trainers. Several states have adopted partial measures: some require athletic trainers at football games, others require emergency action plans that implicitly assume trainer availability, and a growing number have passed legislation in response to student-athlete deaths that could have been prevented by on-site medical professionals.

In Maryland, the Elijah Gorham Act, passed after the death of a student-athlete who collapsed during practice at a school without a trainer, requires schools to develop venue-specific emergency action plans for defibrillator use, heat acclimatization, and coordination of care. The school district hired its first certified athletic trainer as part of a settlement agreement, and committed to placing trainers at every high school offering interscholastic athletics.

Michigan has no state mandate requiring athletic trainers in secondary schools. The Michigan Athletic Trainers' Society and the National Athletic Trainers' Association have advocated for increased coverage, but policy action has been limited.

## **4. Workforce and Economic Barriers**

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The athletic trainer shortage is not solely a demand problem. Supply-side constraints include compensation levels that fall below those available in clinical, collegiate, and professional sports settings. According to the Bureau of Labor Statistics, the average athletic trainer salary in 2024 was approximately \$60,250 per year, but secondary school

trainers, particularly in rural areas, often earn significantly less. A 2024 survey by NATA and ACSM found that low pay was among the top reasons for the collegiate athletic trainer shortage, and the problem is more acute at the secondary school level.

Rural districts face a compounding challenge: they cannot offer competitive salaries, they are located far from the clinical facilities where many trainers prefer to build careers, and their small student bodies make it difficult to justify a full-time position. Some districts have addressed this through shared-services arrangements with local hospitals, health systems, or neighboring districts, but these models depend on the availability and willingness of regional medical providers to extend services to schools.

## 5. Policy Recommendations

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The evidence presented in this brief supports the following policy recommendations. Only one state (Hawaii) currently requires public high schools to employ athletic trainers. Michigan has the opportunity to lead regionally by addressing the rural trainer gap through targeted, cost-effective interventions:

- The state should establish dedicated funding mechanisms for athletic trainer employment in small and rural districts. Suburban schools are 3.55 times more likely to have athletic training services than rural schools, and large schools are 18.5 times more likely than small schools. Alignment with existing education funding formulas or competitive grant programs could close this gap without creating a new spending mandate.
- The Legislature should authorize and incentivize shared-services models that allow regional health systems, hospitals, or multi-district cooperatives to provide athletic training coverage across multiple small schools. Rural emergency response times average 13.6 minutes compared to 7.0 minutes in urban areas, making on-site athletic trainers a critical safety bridge in communities where the nearest hospital may be 30+ minutes away.
- Minimum standards for emergency action plans in secondary school athletics programs, incorporating athletic trainer roles where available and defining alternative protocols where trainers are absent.
- Athletic trainers should be formally integrated into school-based behavioral health referral pathways under MCL 388.1631n. Research increasingly identifies athletic trainers as frontline behavioral health observers. In schools where they are present, they are often the only health care provider student-athletes encounter regularly. Connecting the trainer gap to the broader \$106.5 million Section 31n investment maximizes both funding streams.
- Data collection requirements that track athletic training coverage by school, district, and geographic locale, enabling evidence-based resource allocation decisions.

- Workforce pipeline initiatives that incentivize athletic training graduates to serve in rural and underserved school settings, potentially through loan forgiveness, stipend programs, or clinical rotation requirements.

These recommendations are presented as nonpartisan, evidence-based conclusions grounded in the research documented in this brief and comparative analysis of state-level athletic trainer requirements nationwide.

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### About Northstar Civic Institute

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